

Unplanned Transfers SOP (BHS and TCU)

Scope

Site	Department, division, or operational area	Applicable to
Bentley Health Service (BHS) and Bidi Wungen Kaat (Transitional Care Unit [TCU])	General and Mental Health	All staff

Purpose

This standard operating procedure (SOP) aims to establish minimal practice standards for the unplanned transfer of patients out of Bentley Health Service (BHS) and Bidi Wungen Kaat (Transitional Care Unit [TCU]).

Patients may be transferred for the following reasons:

- Patient deterioration relating to physical and/or mental health requiring tertiary care
- Neonatal complications at or after birth (refer **Midwifery Birth Centre: Transfer/Escort of a Neonate SOP**)
- Diagnostics procedures unavailable at BHS and TCU
- Other clinical concern.

The *Unplanned Transfer Form* is to be completed to allow review of an unexpected transfer to another hospital or tertiary facility, allowing the organisation to assess the care interventions leading up to the transfer and ensure the processes followed support safe transport and care of the patient.

- All BHS/TCU patient transfers required for acute deterioration are to be expedited.
- Where transfers for tertiary management are delayed, escalate as follows:
 - in hours: through Divisional executive
 - after hours: to Senior Nurse on Call (SNoC)

Links to relevant documents

- [Unplanned Transfer Form - BHS](#) (Restricted staff access, electronic)
- [Unplanned Transfer Form - TCU](#) (Restricted staff access)
- [Appendix I: BHS - Flowchart for Unplanned Transfers](#)
- [Appendix II: TCU - Flowchart for Unplanned Transfers](#)

Unplanned Transfers SOP (BHS)

Version: 3.1
Endorsed by: Nursing and Site Director BHS 10/06/2024
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Procedure details

BHS

- When a patient is identified for an unplanned transfer, the Treating Team (during hours) or ^{Be}SAFE (Bentley Safety All Hours for Everyone) Clinical Lead is to be notified, if not already in attendance. ^{Be}SAFE can be contacted via switchboard or Computerised Assisted Radio Personnel System (CARPS).
- Once the decision to transfer has been made, the MO on duty or ^{Be}SAFE Clinical Lead will contact the receiving hospital Speciality Team or Emergency Department (ED). Refer to **General Health Admission, Transfer and Discharge SOP –Transfer of Care Section** and the **Mental Health Inpatient Triage to Discharge SOP**.
- The shift coordinator is to notify the patient's next of kin (NoK)/family/personal support person (PSP) of the transfer to another facility.
- Shift coordinator is to complete Enterprise Bed Management (EBM) Interhospital Transfer booking slip. If direct ward transfer, await acceptance on EBM prior to booking transport.
- ^{Be}SAFE Clinical Lead/Mental Health (MH) Clinical Nurse Specialist (CNS) is to book the ambulance for the transfer.
- NUM/Midwifery Manager/ ^{Be}SAFE Clinical Lead to organise appropriate health professionals to escort the patient, if required.
- Shift coordinator/Midwife, in collaboration with allocated/primary nurse, is to ensure all the patient transfer information is documented in the patient's digital medical record (DMR), relevant charts copied in accordance with local ward guidelines and that special equipment is prepared where applicable
 - Refer to the **Medical Records Transfers and Access SOP** for further details of electronic and hard copy documents required for transfer
- Personal belongings and the patient's own medications (POMs) are to be sent with the patient where possible. Any remaining property is to be kept on the ward, or sent to BHS Patient Property during Cashier's hours, until appropriate arrangements can be made for collection.
- For management of POMs refer to **Medications: Patients' Own Medications (POMs) Use in Hospital Policy – Transfer of Patients Own Medications** section.
- Ward staff are to inform the ward clerical staff in hours or Bed Management Systems Operator 2 (BMSO2) after hours, of the transfer to ensure discharge processes have been completed on Web-Based Patient Administration System (webPAS).

TCU

Refer to [Appendix II: TCU – Flowchart for Unplanned Transfers](#) of this SOP for TCU-specific process. Refer also to **Appendix I: Escalation of Care Processes** in the **Bidi Wungen Kaat Centre (TCU) Referral, Admission, Discharge and Transfer Policy** for additional information about escalation of care at TCU.

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Review processes

- The Treating Team, Ward NUM, BeSAFE Clinical Lead (BHS) or shift coordinator (TCU) completes the *Unplanned Transfer Form* (electronic format at BHS) and submits the form to the following senior clinicians, as relevant:
 - Nursing and Site Director (BHS)
 - BHS Deputy Director of Clinical Services
 - Coordinator of Nursing (CoN) for relevant division
 - Treating Consultant
 - Duty BeSAFE Consultant
 - Ward NUM/Midwifery Manager
 - BHS Medical Emergency Team (MET) Coordinator
 - SNoC
- CoN reviews *Unplanned Transfers* to ensure patient care has been appropriate and effective.
- Referral to relevant speciality/personnel will occur if further investigation is required and completion of a Datix Clinical Incident Notification Form initiated via the Datix Clinical Incident Monitoring System (CIMS), if necessary. Refer to the **Clinical Incident Management and Open Disclosure SOP**.
- CoN to review any subsequent investigations/CIMS.

Compliance monitoring

- BeSAFE will provide monthly unplanned transfer reports to Royal Perth Bentley Group (RPBG) Recognising and Responding to Adult Deterioration (RRAD) Committee for review through BHS RRAD subgroup meetings.
- BHS, in line with National Safety and Quality in Health Care Standard 8 (Recognising and Responding to Acute Deterioration), will continue to review all patients who have been transferred to a tertiary facility.

Facilitator

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Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Bidi Wungen Kaat Centre \(TCU\) Referral, Admission, Discharge and Transfer Policy](#)
- [Medications: Patients' Own Medications \(POMs\) Use in Hospital Policy](#)
- [General Health Admission, Discharge and Transfer SOP](#)
- [Documentation NPS](#)
- [Escalation to Consultants Policy](#)
- [SAFE and BeSAFE Policy](#)
- [Medical Records Transfer and Access SOP](#)
- [Midwifery Birth Centre: External Transfer of an Obstetric Inpatient SOP](#)
- [Midwifery Birth Centre: Transfer/Escort of a Neonate SOP](#)
- [Mental Health Inpatient Triage to Discharge SOP](#)
- [Clinical Incident Management and Open Disclosure SOP](#)

EMHS Policy Hub

- [Inter-hospital Patient Transfer Policy](#)

WA Department of Health Policy Hub

- [Clinical Handover Policy](#)

Related forms

- [Unplanned Transfer Form - BHS](#) (Restricted staff access, electronic)
- [Unplanned Transfer Form - TCU](#) (Restricted staff access)
- *Unplanned Presentations to BHS Record MR2*

Legislative requirements, the evidence, and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy Hub.

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 8: Recognising and Responding to Acute Deterioration - 8.2 Clinical Governance and quality improvement to support recognition and response systems, 8.13 The health service organisation has processes for rapid referral to services that can provide definitive management of acute physical deterioration

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References

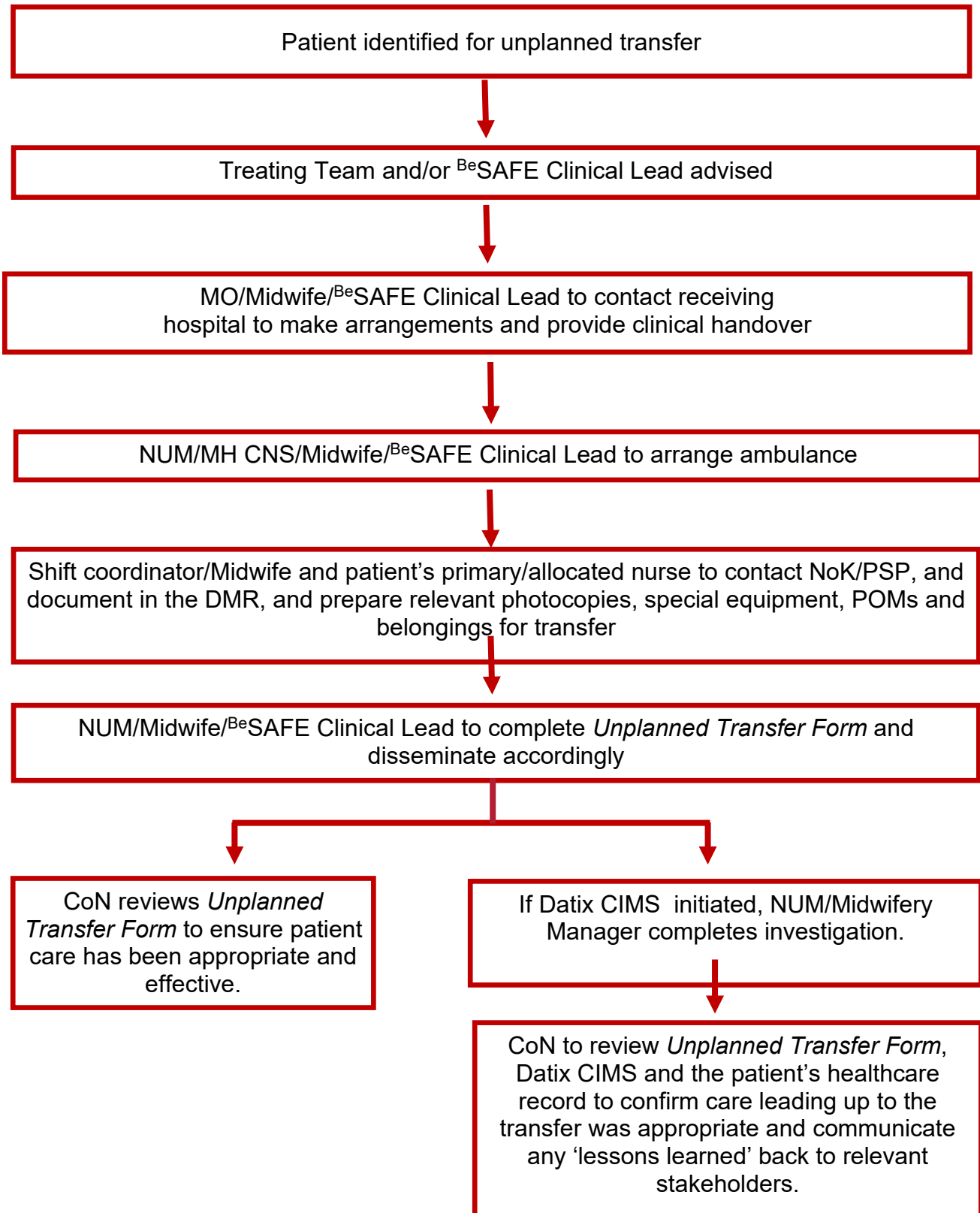
1. Government of Western Australia. (2019). *Clinical Handover Policy*. (DoH MP0095/18 v2.2). Perth. Australia.

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Appendix I: BHS - Flowchart for Unplanned Transfers

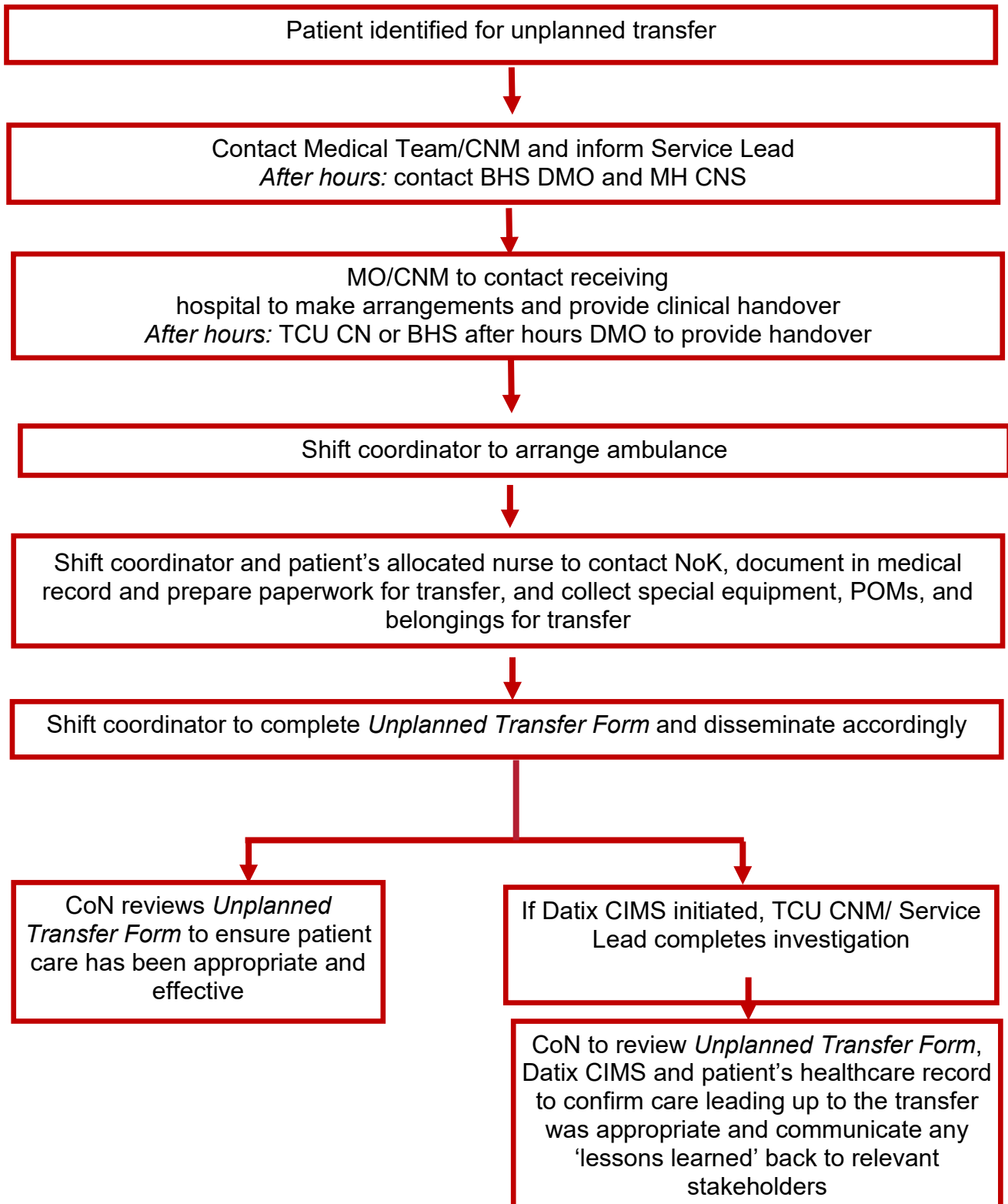


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Appendix II: TCU - Flowchart for Unplanned Transfers



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